



Gu

PCP: [REDACTED] Referring: [REDACTED]
 Appointment Facility: MOORESTOWN Reconstructive Orthopedics, PA

06/22/2023

Progress Notes: [REDACTED] MD

Current Medications

Taking

- Tylenol 8 Hour Arthritis Pain
- Probiotic
- Fish Oil
- Magnesium
- Vitamin C
- Vitamin K2-Vitamin D3
- Zinc

Medication List reviewed and reconciled with the patient

Past Medical History

Hx of right breast cancer 15 years ago.
 Chronic lyme disease.

Surgical History

detached retina left eye 10/2012
 right breast sx 15 years ago
 hysterectomy 2004
 vocal cord sx 12/2008
 LTKR - Purtil 12/21/2020

Family History

Mother Family History: Cancer,
 diagnosed with Cancer

Social History

Tobacco Use:

Tobacco Use/Smoking

Are you a *nonsmoker*

Smoking

Are you a former smoker? No

Allergies

N.K.D.A.

Hospitalization/Major

Diagnostic Procedure

Denies Past Hospitalization

Review of Systems

General/Constitutional:

Change in appetite Denies. Weight

Reason for Appointment

1. Right knee pain

Assessments

1. Primary osteoarthritis of right knee - M17.11

Treatment

1. Primary osteoarthritis of right knee

IMAGING: Knee the works R

Clinical Notes:

We discussed options for treatment available for knee arthritis. These include injections, bracing, strengthening exercises, medications, and weight reduction, as well as doing nothing. We also discussed knee replacement as an option should the pain and dysfunction be at a point where it is affecting quality of life.

Please note: This EHR document was generated by the provider using dictation/free text. Please contact our office if there are any questions at (609)267-9400.

2. Others

Notes: For now, we have agreed that she is doing well and there is no reason to intervene with steroid injections or surgery. She is always welcome to come back to see one of the PAs or myself for cortisone shot if needed but right now she declined given that she is doing okay. She will continue working on her range of motion but understands that keeping the strength up and keeping her weight down are beneficial. We can try cortisone if needed but we would not try Visco again. While at some point she will need surgery again stressed to her that routine x-rays to see if she was ready for surgery does not matter to me and what matters more as her quality of life and she gets this. She also tells me that she did get at least 80% relief in her left knee and although it stiff it is very good for her and

Progress Note: [REDACTED] MD 06/22/2023

gain Denies. Weight loss Denies. Pain at night Admits. Difficulty sleeping Denies. Numbness Admits.

Allergy/Immunology:

Seasonal allergies Admits.

Ophthalmologic:

Eye problems Denies.

ENT:

Vertigo Denies.

Endocrine:

Thyroid problems Denies.

Respiratory:

Shortness of breath Denies.

Asthma Denies.

Cardiovascular:

Palpitations Denies. Chest pain Denies.

Gastrointestinal:

Reflux Denies.

Hematology:

Anemia Denies.

Genitourinary:

Frequent urination Denies.

Musculoskeletal:

Swollen joints Denies.

Peripheral Vascular:

Pain/cramping in legs after exertion Denies.

Skin:

Discoloration Denies.

Neurologic:

Tingling/Numbness Admits.

Migraines Denies. Weakness Denies.

Psychiatric:

Anxiety Denies. Depression Denies.

much better than before surgery and explained to her that that is what we expect after knee replacement and not a "normal" knee. She is very appreciative the discussion and will continue her exercise program and come back as needed. We did briefly discussed that months time comes as long as she has help she is very healthy and could have surgery as an outpatient but would be managed very differently than it was at Rothman.

Follow Up

prn

History of Present Illness

FOLLOW UP:

FOLLOW UP

Are your symptoms *Unchanged*

Are there any new injuries or limitations? *No*

Are you currently in physical therapy? *No*

Have there been any changes in your medical history? *No*

Have you had a total knee replacement in the past year? *No*

MIPS:

MIPS

Please select your sex: *Female*

Age: *64 years old or older*

Do you currently use any form of tobacco product? i.e. smoking, chew, vape, etc. *No*

Do you suffer from urinary leakage? *No*

Do you have an advanced directive? (i.e. Living Will, POA, DNR) *Yes*

Please select the type(s) of advanced directive you have: *Living*

Will, Designated Decision Maker/POA

Please select from the following options: *I have an advanced directive but have NOT provided a copy to Reconstructive Orthopedics*

Urinary Incontinence

Do you suffer from urinary leakage? *Absent*

Tobacco Use

Tobacco Use *Negative*

BMI

MIPs completed by: *Normal BMI, AG*

HP Advanced Directive

Do you have an advanced directive? (e.g. Living will, POA, DNR) *Yes*

Please select the type(s) of advanced directive you have: *Living Will, Designated Decision Maker/POA*

Please select from the following options: *I have an advanced directive but have NOT provided a copy to Reconstructive Orthopedics.*

History of Present Illness:

She returns today for follow-up. I saw her well over a year ago for her right knee. She took for follow-up mostly just to check in. She tells me she is actually doing okay. She had Visco injections Actros SR she tells me that did not really help a whole lot. She reports and interested in doing it again. She does not recall having steroid injections. We discussed also today that when she had her left knee done at Rothman she does not feel she was at all prepared for how challenging it would be in PT was not set up and she had a lot of difficulty with recovery but at this point does feel very happy with her left knee replacement. She is working with a personal trainer and exercising and feels that is keeping her strength up in both the right and

the left legs. As far as the right knee she tells me that she does not have marked pain most of the time and right now and even the same take an injection if we needed to do 1 but has not had a cortisone shot knows that that would be an option as well. She is not using a cane or a walker. She does live alone and came alone. She simply came in to "check-in" and see how things look. She also want to be sure she get a picture of her current x-ray.

Vital Signs

Ht: 65 in, Wt: **131.6 lbs**, BMI: **21.9 Index**.

Examination

Right knee:

INSPECTION: mild effusion.

LEG EXAMINATION: neurovascularly intact distally.

PALPATION: joint line tenderness present.

KNEE RANGE OF MOTION: passive (in degrees) 5-120.

STRENGTH: mildly decreased, pain inhibited.

LIGAMEN TOUS LAXITY (NEG): all ligaments appear stable.

TESTS: crepitus present.

HIP RANGE OF

MOTION: within normal limits, no pain on range of motion.

GAIT: walks well with a mild limp.

Left knee:

INSPECTION: no significant swelling, no deformity.

LEG EXAMINATION: neurovascularly intact distally.

ALIGNMENT: neutral.

WOUNDS: old healed total knee replacement scar.

PALPATION: no effusion palpated, no specific tenderness on palpation.

KNEE RANGE OF MOTION: 0-115 with 5 deg ext lag.

STRENGTH: 5/5 quad strength.

LIGAMEN TOUS LAXITY (NEG): all ligaments appear stable.

HIP RANGE OF MOTION: no pain on range of motion.

General Examination:

PSYCH: alert, oriented.

Imaging Studies:

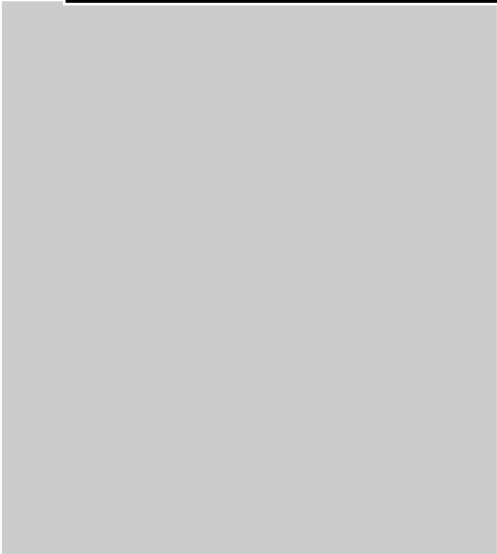
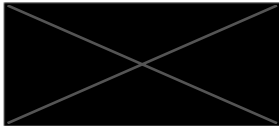
X-rays taken today to evaluate right knee pain and personally reviewed by myself included standing and flexed as well as patellar views of both knees and a lateral of the right knee. The left cemented knee replacements well aligned with no sign of wear or loosening. This is unavailable final views. The left knee looks good. The right knee is bone-on-bone laterally as seen previously with spurring and sclerosis. Mild retropatellar arthritic changes are noted on the right and a resurfaced patella on the left.

Procedure Codes

73564 X-ray Knee,complete, 4 OR MORE Views

73560 X-ray Knee,1 or 2 views

 **DOB:**  (67 yo F) **Acc No.**  **DOS:** 06/22/2023

Electronically signed by  **MD on 06/22/2023 at**
05:17 PM EDT

Sign off status: Completed

MOORESTOWN Reconstructive Orthopedics, PA

Progress Note: 

Note generated by eClinical


☒ Guarantor

Insurance: HORIZON BCBS NJ PPO

PCP: DO Referring: DO
Appointment Facility: VOORHEES Reconstructive Orthopedics Pa

11/17/2021

, MD

Current Medications

Taking

- Probiotic
- Fish Oil
- Magnesium
- Vitamin C
- Vitamin K2-Vitamin D3
- Zinc

Discontinued

- Mobic 15 MG Tablet 1 tablet Orally Once a day
- Medication List reviewed and reconciled with the patient

Past Medical History

H of right breast cancer 15 years ago.
Chronic lyme disease.

Surgical History

detached retina left eye 10/2012
right breast sx 15 years ago
hysterectomy 2004
vocal cord sx 12/2008
LTKR - Purtil 12/21/2020

Family History

Mother Family History:
Cancer, Osteoporosis/Osteopenia,
diagnosed with Cancer
Siblings Family History:
Osteoporosis/Osteopenia

Social History

Tobacco Use:

Tobacco Use/Smoking
Are you a *nonsmoker*
Smoking

Are you a former smoker? *No*

Drugs/Alcohol:

Alcohol Screen (Audit-C)
Did you have a drink containing
alcohol in the past year? *No*
Drugs

Do you use any illegal substances? *No*

Miscellaneous:

Reason for Appointment

1. Right knee pain

Assessments

1. Primary osteoarthritis of right knee - M17.11

Treatment

1. Primary osteoarthritis of right knee

IMAGING: Knee the works R

Notes: Knee Arthritis: Exercises material was printed.

Clinical Notes:

We discussed options for treatment available for knee arthritis. These include injections, bracing, strengthening exercises, medications, and weight reduction, as well as doing nothing. We also discussed knee replacement as an option should the pain and dysfunction be at a point where it is affecting quality of life. While she understands that eventually she will need the right knee replaced, she has not had any conservative treatment and we discussed the difference between cortisone and Visco injections but she decided to hold off on either. I did review with her that she has a little bit of an extensor lag in the left side this could still be improved given that passively she comes straight. We went over heel props and other exercises for this. As far as the right knee goes if she opts for injection she is aware that the PAs are booked out a few weeks but she can call and get an injection. She wants Visco injections we can order those and hold off on joint placement as long as possible. I did give her a clinician Gollotto exercise regimen to continue to keep her strength up.

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2. Others

Notes: The patient was screened as per office protocol before being seen today. The patient accepted the risk of being seen in person today given the current COVID-19 pandemic. Extra care and precaution was taken as indicated via handwashing/wearing gloves and/or mask to decrease risk of infection to patient and/or myself. I also attempted to maintain

Living Arrangement
Do you live alone? *Yes*

Allergies

N.K.D.A.

Hospitalization/Major

Diagnostic Procedure

Denies Past Hospitalization

Review of Systems

General/Constitutional:

Change in appetite Denies. Weight gain Denies. Weight loss Denies. Pain at night Admits. Difficulty sleeping Admits. Numbness Denies.

Allergy/Immunology:

Seasonal allergies Admits.

Ophthalmologic:

Eye problems Denies.

ENT:

Vertigo Denies.

Endocrine:

Thyroid problems Denies.

Respiratory:

Shortness of breath Denies. Asthma Denies.

Cardiovascular:

Palpitations Denies. Chest pain Denies.

Gastrointestinal:

Reflux Denies.

Hematology:

Anemia Denies.

Genitourinary:

Frequent urination Admits.

Musculoskeletal:

Swollen joints Admits.

Peripheral Vascular:

Pain/cramping in legs after exertion Denies.

Skin:

Discoloration Denies.

Neurologic:

Tingling/Numbness Denies. Migraines Denies. Weakness Denies.

Psychiatric:

Anxiety Denies. Depression Denies.

appropriate distance between the patient and myself during the visit/conversation.

Follow Up

prn

History of Present Illness

NEW/EXT PATIENT:

NEW/EXT PATIENT

Is your reason for today's visit for a *Chronic ongoing condition*

Is your injury due to a motor vehicle accident? *No*

Is your injury under workers compensation? *No*

Are you right or left hand dominant? *Right*

Do you currently work? *Yes*

Do you work? *Full Time*

What is your occupation? *School nurse*

MIPS:

MIPS

Please select your sex: *Female*

Age: *64 years old or older*

Do you currently use any form of tobacco product? i.e. smoking, chew, vape, etc. *No*

Do you suffer from urinary leakage? *Yes*

Do you have an advanced directive? (i.e. Living Will, POA, DNR) *No*

Advanced directives are designed to respect patient's autonomy and determine his/her wishes about future life sustaining medical treatment if unable to indicate wishes. It is recommended that you discuss an advanced directive with your provider. *I acknowledge that I have been made aware of the importance of an advanced directive and offered the opportunity to discuss my advanced care plans with my clinician.*

Urinary Incontinence

Do you suffer from urinary leakage? *Present*

Tobacco Use

Tobacco Use *Negative*

BMI

Above normal BMI follow up plan *Patient referred to PCP for abnormal BMI*

MIPs completed by: *AM*

HP Advanced Directive

Do you have an advanced directive? (e.g. Living will, POA, DNR) *No*

Advanced directive are designed to respect a patient's autonomy and determine his/her wishes about future life sustaining medical treatment if unable to indicate wishes. It is recommended that you discuss an advanced directive with your provider. *I acknowledge that I have been made aware of the importance of an advanced directive and offered the opportunity to discuss my advanced care plans with my clinician.*

Total Joint Hip/Knee:

HPI

List problem areas *Right knee pain*

Duration of symptoms *1 year*

Was there an injury? *No*

Swelling *No*

Catching or locking *Yes*

Instability or giving way *No*
 Snap or pop *Yes*

Pain

Are you in pain? *Yes*

Location *Anterior*

How severe is your pain? 1= No pain, 10= Worst pain imaginable *9*

When most severe

Is there pain with motion? *Yes Prolonged periods of sitting*

Numbness/tingling *No*

Radiation *No*

Prior Treatments

NSAID *Improved Advil*

Ice *No benefit*

Heat *No benefit*

Brace *No benefit Compression sleeve*

Physical therapy/Home exercise program *Did not try*

Injections/Visco supplements *Did not try*

History of Present Illness:

This is a pleasant 66-year-old who came in and lives alone. She had her left knee replaced at Rothman just under a year ago and tells me she was not prepared for how difficult it was. She tells me that she was not offered any conservative treatment and was told she should have her knee replaced. She is having trouble sleeping at night and so she went ahead with it. She is a school nurse and unfortunately took many months to feel that she can get back to work. She takes Advil on a as needed basis and has never had any injections in the right knee. She has pain with weightbearing and ADLs. She was not offered any treatment in the left knee other than surgery. She was taking Celebrex at one time but now only takes Advil on a as needed basis. She is interested in what other options she would have. Her left knee ultimately turned out very well but she like to hold off on the right side as long as possible.

Vital Signs

Ht 5 ft 5 in, Wt 149.6 lbs, BMI **24.89 Index**, Ht-cm 165.1 cm, Wt-kg 67.86 kg.

Examination

Right knee:

INSPECTION: mild effusion.

LEG EXAMINATION: neurovascularly intact distally.

PALPATION: joint line tenderness present.

KNEE RANGE OF MOTION: passive (in degrees) 10-120.

STRENGTH: mildly decreased, pain inhibited.

LIGAMENTOUS LAXITY (NEG): all ligaments appear stable.

TESTS: crepitus present.

HIP RANGE OF MOTION: within normal limits, no pain on range of motion.

GAIT: walks well with a mild limp.

Left knee:

INSPECTION: no significant swelling, no deformity.

LEG EXAMINATION: neurovascularly intact distally.

ALIGNMENT: neutral.

WOUNDS: old healed total knee replacement scar.

PALPATION: no effusion palpated, no specific tenderness on palpation.

KNEE RANGE OF MOTION: PROM:0-115, 5 deg ext lag.
STRENGTH: 5/5 quad strength.
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HIP RANGE OF MOTION: no pain on range of motion.

General Examination:

PSYCH: alert, oriented.

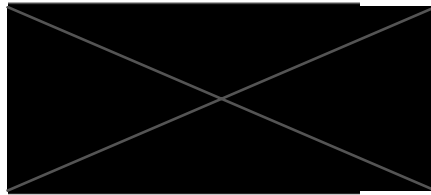
Imaging Studies:

X-rays taken today in our office and personally reviewed by me to evaluate right knee pain included Standing AP and 45 flexed views of both knees as well as lateral and Merchants views. There is subchondral sclerosis present as well as spurring and the right knee is bone on bone laterally with mild patellar arthritis. The cemented DePuy L TKA is well aligned on the frontal views with non loosening seen.

Procedure Codes

73564 X-ray Knee,complete, 4 OR MORE Views

73560 X-ray Knee,1 or 2 views



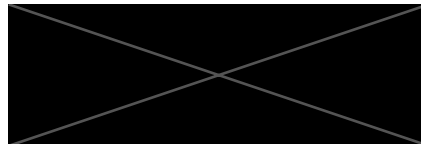
Electronically signed by , MD on 11/17/2021 at 04:54 PM EST

Sign off status: Completed

Addendum:

12/27/2021 08:27 AM  > Addendum added for correct documentation of advanced directive to the right chart panel.

VOORHEES Reconstructive Orthopedics Pa



Progress Note:  MD 11/17/2021

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